INTERNSHIP REPORT

Submitted

For the Partial Fulfilment of the Requirement for the Degree of Bachelor of _____ with _____ (Major and Minor Subjects) under **Magadh University**

Submitted to

The Department of _____, _____ **(College Name)**

[Paste College Logo here]

Submitted by

Name : _____

Semester : _____

Programme FYUGP : _____

University's Registration No. : _____

College Roll No. : _____

Year : _____

RK NEXORA
CAMPUS TO CAREER
A Unit of RKNexora Private Limited.

INTERNSHIP REPORT

Topic - Hospital & Healthcare Administration

Submitted to the Head of Department, _____ (College Name), in partial fulfilment of the requirements for the Degree of _____, Magadh University

Name of Student : _____

Programme : _____

University Registration No. : _____

College Roll No. : _____

Internship Registration No. : _____

Name of the IPO / Agency : RKNEXORA PRIVATE LIMITED

Internship Period : _____ to _____

Semester : _____

Year / Session : _____

RKNEXORA PRIVATE LIMITED
RKNEXORA
PRIVATE LIMITED
GAYA, BIHAR - 805236, INDIA

_____ _____

Signature of Student **Seal & Signature of IPO**

ACKNOWLEDGEMENT

I express my sincere gratitude to RK NEXORA, RKNEXORA PRIVATE LIMITED, _____ (College Name), my faculty mentors and the internship supervisor, _____, for their guidance and support. The structured activities, learning resources and regular feedback helped me complete this internship in Hospital & Healthcare Administration successfully.

I am thankful to the Head of the Department and the college mentor for their encouragement and for guiding me in preparing this report. I am also grateful to the management of RKNEXORA PRIVATE LIMITED for providing a professional learning environment and the opportunity to learn and apply my knowledge in a practical setting. The concept sessions, hands-on tasks with sample hospital forms, and the supervisor's patient and detailed feedback helped me understand how a hospital works behind the scenes.

Finally, I thank my family and peers for their constant encouragement throughout the programme.

_____

Name: _____

University Registration No. - _____

INTERNSHIP COMPLETION CERTIFICATE

The Internship Completion Certificate issued by RKNEXORA PRIVATE LIMITED for the internship in Hospital & Healthcare Administration is to be attached on this page.

[Attach the original / printed copy of the QR-verified Internship Completion Certificate here]

Certificate No. _____

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PREFACE

Internships are an important part of the undergraduate programme, as they allow students to see how classroom learning is applied in real organisations. The Four-Year Undergraduate Programme (FYUGP), introduced in line with the National Education Policy 2020, gives special importance to experiential learning, skill development and employability. This report has been prepared as part of the internship requirement of Magadh University and the Higher Education guidelines applicable in Bihar.

The report describes the internship undertaken in Hospital & Healthcare Administration at RKNEXORA PRIVATE LIMITED, the plan followed, the work carried out, and the learning gained. Healthcare is one of the largest and fastest-growing service sectors in India, and it needs not only doctors and nurses but also trained administrators who can manage registration, records, billing, stores, quality and patient relations. This internship offered a first-hand introduction to that side of healthcare.

The report has been arranged in the order suggested by the University, beginning with the cover page and acknowledgement and ending with the conclusion and attendance sheet. It is hoped that the report gives a clear and honest account of the internship experience.

EXECUTIVE SUMMARY

This report documents the internship completed in Hospital & Healthcare Administration at RKNEXORA PRIVATE LIMITED (RK NEXORA - 'Campus to Career'). The programme required a minimum of 120 hours, completed over about four to five weeks (28 working days) with daily sessions of four to five hours, and was supported by an attendance log, supervisor guidance and review.

Background. A hospital is a complex organisation in which clinical care and administrative support must work together. Patients depend not only on doctors and nurses but also on well-organised registration, records, billing, pharmacy, quality systems and grievance handling. Hospital and healthcare administration is the discipline that plans, coordinates and monitors these services so that care is safe, timely, ethical and efficient. For a student from any discipline, the internship offers a way to see how ideas of management, records, accountability, law and ethics operate in a healthcare setting.

Objectives. The internship aimed to (i) build a structured understanding of hospital administration, (ii) study the working of hospital departments and patient flow, (iii) learn patient registration, OPD / IPD administration, billing and insurance claims, medical records and pharmacy administration, (iv) understand quality, patient safety, NABH standards, healthcare laws and ethics, and (v) develop communication, documentation and professional discipline.

Work carried out. The student first studied the structure of a hospital and the journey of a patient through it, then worked through patient registration and records, OPD / IPD management, billing and insurance claims, medical records (MRD), hospital inventory and pharmacy, quality and patient safety, NABH standards, healthcare laws and ethics, and patient feedback and grievances. Each topic had a concept session followed by a hands-on task using sample forms and dummy records. The later part of the programme was devoted to a case study, a mock practical exercise, supervisor review and preparation of this report.

Project. The work was tied together in a project titled *Hospital Workflow & Patient Records Management*, in which OPD / IPD workflow was studied and patient registration, billing and medical record documentation were practised using sample data.

Method. A blended method was followed - learn, practise, document and review - supported by a daily attendance log with day-wise remarks and regular feedback from the supervisor.

Learning gained. The student gained a clear understanding of how hospital administration supports patient care, practical familiarity with registration, billing, records and stock documentation, awareness of quality standards (NABH) and patient rights, and improved communication, empathy and professional discipline.

Key observations. Accurate patient records and billing are the foundation of reliable hospital administration; patient information must always be kept confidential; and clear processes and checklists improve patient safety and patient satisfaction.

Conclusion. The internship met its objectives and contributed to the student's academic understanding, employability and confidence. The report follows the University's suggestive format: cover page, acknowledgement, certificate, contents, preface, executive summary, about the agency, plan and execution, learning outcome, conclusion and attendance sheet.

ABOUT THE AGENCY

RK NEXORA, a unit of RKNEXORA PRIVATE LIMITED, provides structured internship and skill-development programmes under the theme 'Campus to Career'. The organization focuses on academic-industry alignment, practical exposure, assessment, documentation and employability-oriented learning.

Particulars	Details
Name of Agency	RKNEXORA PRIVATE LIMITED
Programme / Unit	RK NEXORA - Campus to Career
Registered Office	C/O Mukesh Kumar, Khiri, PS Atri, Jethiyan, Atri, Gaya - 805236, Bihar, India
CIN	U85499BR2026PTC086757
Email	rknexorapvtltd@gmail.com
Website	www.rknexora.org
Internship Topic	Hospital & Healthcare Administration

1. Vision

To support students in developing practical, professional and industry-relevant capabilities.

2. Mission

To deliver structured learning experiences that connect academic knowledge with workplace expectations.

3. Activities of the Agency

1. Structured internship programmes aligned with university and higher-education requirements.
2. Skill-development learning modules, practical activities and assignments.
3. Attendance tracking, supervision, assessment and documentation.
4. Employability-oriented guidance connecting campus learning to career readiness.

4. Programme Structure

The internship programme is organised around a fixed number of learning hours, completed over a defined period. Each intern is registered on the internship portal with a unique registration number, is assigned a supervisor, follows a day-wise plan and maintains an attendance log. Learning is organised into three phases: **Orientation** (Days 1-2), **Learning and Practice** (Days 3-22, ten sub-topics of two days each: one concept day and one hands-on day) and **Project and Review** (Days 23-28), followed by a concluding review.

5. Role of the Supervisor

The internship supervisor assigned by the agency guided the student through the programme, explained concepts, assigned practice tasks, reviewed the daily log and gave feedback. The supervisor also verified attendance and reviewed the student's work at the end of the programme.

PLAN AND EXECUTION

1. Objectives

The internship was planned around the following objectives, each linked to a part of the programme:

1. Understand Hospital & Healthcare Administration - role, structure and functions of a hospital (Week 1).
2. Study hospital departments and patient flow - clinical, para-clinical and support services (Week 1).
3. Learn patient-care administration - registration, OPD / IPD management, billing, insurance claims and medical records (Weeks 1-2).
4. Understand support services - hospital inventory and pharmacy administration (Week 3).
5. Learn quality, law and ethics - patient safety, NABH standards, healthcare laws, ethics and grievance handling (Weeks 3-4).
6. Develop professional discipline - communication, empathy, confidentiality, documentation and report writing (throughout).

2. Methodology

The programme followed a blended method made up of four connected steps, repeated for each topic:

Learn. Each topic began with a concept session in which the supervisor explained the purpose of the function, its main records and how it connects with other hospital services. Notes were prepared in the student's own words.

Practise. The concepts were then applied in guided hands-on tasks - filling sample registration and admission forms, preparing sample bills and insurance claim checklists, arranging dummy medical records, maintaining sample stock registers and completing safety checklists. Only sample or dummy data were used, so that no real patient information was handled.

Document. Every day's work was recorded in the internship attendance log with a short remark, and processes were written up as documentation so that the work could be reviewed.

Review. The supervisor reviewed the work, pointed out errors and suggested improvements. Review days, a case study and a mock practical exercise helped to consolidate the learning.

3. Understanding Hospital & Healthcare Administration - Concepts Studied

3.1 What is hospital administration. Hospital administration is the planning, organisation, coordination and control of all the services of a hospital - both the clinical services that treat patients and the non-clinical services that support them - so that quality care is delivered safely, ethically and efficiently. Its main functions are planning, organising, staffing, directing, coordinating and controlling resources such as people, money, equipment, medicines and information.

3.2 Types of hospitals. Hospitals can be classified by ownership (government, private, trust / charitable, corporate), by level of care (primary: sub-centres and Primary Health Centres; secondary: Community Health Centres and district hospitals; tertiary: medical college hospitals and super-speciality institutes), and by size and speciality (general, maternity, eye, cancer, etc.). India's public health system follows this three-tier structure, and patients are often referred upward from one level to the next.

3.3 Structure of a hospital. A hospital's services are commonly grouped into (i) clinical departments such as medicine, surgery, paediatrics, obstetrics and emergency, (ii) para-clinical or diagnostic services such as laboratory, radiology and blood bank, (iii) support services such as pharmacy, stores, housekeeping, laundry, dietary, CSSD (sterilisation), biomedical engineering and security, and (iv) administrative services such as registration, billing, medical records, human resources and accounts.

A typical hospital is organised in levels, as shown below.

Level	Position	Main role
Governance	Governing body / Trust / Board	Sets policy, approves budget and ensures legal compliance
Top management	Medical Director / CEO / Medical Superintendent	Overall control of clinical and administrative services
Middle management	Heads of departments, Nursing Superintendent, Hospital Administrator, Finance and HR managers	Run departments, supervise staff and coordinate services
Operational staff	Doctors, nurses, technicians, front office, billing, MRD and store staff	Deliver care and carry out daily administrative tasks

3.4 Patient flow. A patient's journey - registration, consultation, investigations, treatment or admission, billing, discharge and follow-up - passes through several departments. Administration ensures that each step is connected to the next, without delay or loss of information. Poor patient flow shows up as long waiting times, repeated paperwork and patient dissatisfaction.

3.5 Registration, UHID and records. Registration creates the patient's identity in the hospital system. A Unique Hospital ID (UHID) is allotted so that all visits of the same patient are linked to one record. Hospitals increasingly link records with the ABHA (Ayushman Bharat Health Account) number under the Ayushman Bharat Digital Mission. Medical records form the legal and clinical history of a patient; they must be accurate, complete, legible, stored safely and shared only with authorised persons. Respect for patient privacy and dignity is a basic ethical duty of everyone working in a hospital.

3.6 Health insurance and financing. Patients pay through out-of-pocket payment, private insurance (cashless or reimbursement), employer schemes, or government schemes such as Ayushman Bharat PM-JAY and CGHS. A Third Party Administrator (TPA) or the insurer processes claims; hospitals must therefore keep correct documents, package rates and approvals.

3.7 Quality and patient safety. Quality systems, checklists and accreditation standards help hospitals reduce errors, prevent infections and improve patient satisfaction. In India, hospital quality is commonly assessed against the standards of the National Accreditation Board for Hospitals & Healthcare Providers (NABH).

3.8 Key performance indicators (KPIs). Administrators track numbers to judge how well a hospital is working:

Indicator	Meaning / Formula	Why it matters
Bed Occupancy Rate (BOR)	(Inpatient days / (Beds x Days in period)) x 100	Shows how fully beds are used
Average Length of Stay (ALOS)	Total inpatient days / Total discharges	Shows efficiency of treatment and bed use
Bed Turnover Rate	Discharges / Number of beds	Shows how many patients each bed serves
OPD Waiting Time	Time from registration to consultation	Reflects patient experience
Claim Rejection Ratio	Rejected claims / Claims submitted x 100	Shows quality of billing documentation
Patient Satisfaction Score	Average rating from feedback forms	Shows service quality

3.9 Limitations and care needed. Hospital administration depends on trained staff, clear procedures, reliable records and teamwork; weak documentation or poor coordination can delay care and affect patient safety.

4. Week-wise Plan and Execution

Week	Days	Activities
Week 1	Days 1-6	Orientation and foundations: company induction, introduction to hospital administration, study of hospital departments and workflow, and patient registration and records.
Week 2	Days 7-12	Patient care administration: OPD / IPD management, billing and insurance claims, and medical records (MRD), each with a concept session and a hands-on task.
Week 3	Days 13-18	Support services and quality: hospital inventory and pharmacy, quality and patient safety, and NABH standards.
Week 4	Days 19-24	Law, feedback and application: healthcare laws and ethics, patient feedback and grievances, a case study exercise and a mock practical exercise.
Week 5	Days 25-28	Project closure: supervisor review and feedback, internship report drafting, presentation preparation and final review.

5. Week-wise Narrative

Week 1: The first week was devoted to orientation and foundations. After company induction, the student was introduced to hospital administration and studied the departments of a hospital and the flow of a patient through them. The week ended with a study of patient registration and the records created at the front desk.

Week 2: In the second week the study moved to OPD and IPD management - appointments, consultation, admission, transfer and discharge - followed by billing and insurance claims, including estimates, interim bills, pre-authorisation and claim documentation, and the arrangement and safe keeping of medical records (MRD).

Week 3: The third week focused on support services and quality: administration of hospital inventory and pharmacy (indents, stock registers, expiry checks), quality and patient safety (checklists, infection control, incident reporting) and the main standards of NABH.

Week 4: In the fourth week the student studied healthcare laws and ethics, including patient rights, informed consent and confidentiality, and learned how patient feedback and grievances are collected and resolved. A case study exercise and a mock practical exercise were completed in this week.

Week 5: The last week was devoted to project closure: supervisor review and feedback, drafting of the internship report, preparation of a presentation and final review before submission.

6. Topic-wise Learning in Detail

6.1 Hospital Departments and Workflow

Concept: Explains the clinical, para-clinical, support and administrative departments of a hospital, and how they coordinate to deliver care. The emergency department, OPD, wards, operation theatre, ICU, laboratory, radiology, pharmacy and stores were studied as connected units.

Practice carried out: Studying a hospital organisation chart; mapping the flow of a sample patient across departments.

6.2 Patient Registration and Medical Records (MRD)

Concept: Registration creates the patient's identity and record; the Medical Records Department stores, retrieves and protects records. Concepts included UHID, consent forms, filing systems (serial, terminal-digit and alphabetical numbering), record retrieval, retention and confidentiality. Indian Medical Council

professional conduct regulations require indoor patient records to be kept for a minimum period (at least three years), and hospitals often keep them longer.

Practice carried out: Filling sample registration forms; arranging dummy case files; practising record retrieval and filing.

6.3 OPD / IPD Management

Concept: The Out-Patient Department (OPD) handles patients who are not admitted; the In-Patient Department (IPD) handles admitted patients. Concepts included appointment, consultation, admission, bed allotment, transfer and discharge, and the discharge summary. The role of triage in emergency (sorting patients by urgency) was also understood.

Practice carried out: Preparing a sample OPD flow chart; completing a sample admission and discharge checklist.

6.4 Billing and Insurance Claims

Concept: Billing converts services into charges; insurance claims recover charges from insurers or TPAs. Concepts included estimates, interim and final bills, package rates, pre-authorisation, claim documents, query reply and settlement, and common reasons for claim rejection (missing documents, wrong diagnosis or procedure details, non-covered items).

Practice carried out: Preparing a sample bill; listing documents required for a sample insurance claim.

6.5 Hospital Inventory and Pharmacy

Concept: Ensures that medicines, consumables and equipment are available when needed. Concepts included indent, purchase, stores, issue, stock register, expiry management (FEFO - first expiry, first out), reorder level and inventory classification (ABC analysis by value, VED analysis by criticality). Reorder level = daily consumption x lead time (+ safety stock).

Practice carried out: Maintaining a sample stock register; checking sample items for expiry and reorder level.

6.6 Quality, Patient Safety, NABH, Laws and Ethics, and Patient Feedback

Concept: Quality systems and standards protect patients and staff. Concepts included infection control (hand hygiene, bio-medical waste segregation), incident reporting, the NABH chapters, patient rights, informed consent, confidentiality and grievance redressal.

Practice carried out: Completing a sample safety checklist; drafting a sample patient-feedback summary and grievance flow.

7. NABH Standards, Laws and Patient Safety - Key Points Studied

NABH chapters. The NABH hospital standards are organised into chapters (as in the 5th edition; the latest edition should be referred to for updates):

Code	Chapter	Focus
AAC	Access, Assessment and Continuity of Care	Registration, admission, assessment, discharge
COP	Care of Patients	Clinical care, emergency, surgical and anaesthesia care
MOM	Management of Medication	Storage, prescribing, dispensing, safe medication use
PRE	Patient Rights and Education	Rights, consent, patient and family education
HIC	Hospital Infection Control	Infection prevention, surveillance, hand hygiene
PSQ	Patient Safety and Quality Improvement	Quality indicators, incident reporting
ROM	Responsibilities of Management	Leadership, governance, ethics

Code	Chapter	Focus
FMS	Facility Management and Safety	Fire safety, equipment, environment
HRM	Human Resource Management	Staff recruitment, training, credentials
IMS	Information Management System	Medical records, data security

Important laws and rules (introductory awareness).

Law / Rule	Relevance to hospitals
Clinical Establishments (Registration and Regulation) Act, 2010	Registration and minimum standards for clinical establishments
Consumer Protection Act, 2019	Patients as consumers; remedies for deficiency in service
Digital Personal Data Protection Act, 2023	Protection of digital personal data, including patient data
Bio-Medical Waste Management Rules, 2016	Segregation, storage and disposal of hospital waste
Drugs and Cosmetics Act, 1940	Control over sale and storage of medicines
Registration of Births and Deaths Act, 1969	Mandatory reporting of births and deaths
Mental Healthcare Act, 2017	Rights of persons with mental illness

Patient safety goals. Correct patient identification, effective communication (including read-back of verbal orders), safe medication practice, safe surgery, prevention of healthcare-associated infection (the five moments of hand hygiene) and prevention of patient falls.

Patient rights. Right to information, right to confidentiality and privacy, right to informed consent, right to a second opinion, right to emergency care and right to make a complaint and be heard.

8. Hospital Process Flows Studied

Process mapping was used to understand how work moves through a hospital and which department is responsible at each step.

Process	Steps
OPD Patient Journey	Registration > Consultation > Investigations (if advised) > Pharmacy / Billing > Follow-up
IPD Patient Journey	Admission > Bed allotment > Treatment > Interim billing > Discharge summary > Final bill > Discharge
Insurance Claim	Pre-authorisation > Treatment > Document collection > Claim submission > Query reply > Settlement
Pharmacy and Stores	Indent > Purchase > Receipt in stores > Issue > Stock register > Reorder review
Patient Grievance	Feedback / complaint received > Recorded > Reviewed > Action taken > Patient informed

9. Worked Examples with Sample (Dummy) Data

Example A: OPD patient journey. To understand how one patient moves through several departments, a simple example was worked through using dummy data. These are illustrative sample entries, not real patient records.

Step	Action	Record created	Department
1	Registration of Sample Patient A (dummy)	UHID and OPD registration slip	Registration / Front desk
2	Consultation	OPD slip with doctor's advice	OPD
3	Investigation advised	Lab / imaging request form	Laboratory / Radiology
4	Billing	Sample bill for consultation and tests	Billing
5	Medicines issued	Pharmacy issue slip; stock register reduced	Pharmacy / Stores
6	Follow-up planned	Follow-up date noted; record filed	OPD / MRD

The example shows why complete registration and careful record-keeping matter: an error at step 1 would carry into the bill, the pharmacy record and the patient's file.

Example B: Sample OPD bill (dummy figures).

Item	Quantity	Rate (Rs.)	Amount (Rs.)
Registration fee	1	50	50
Consultation	1	300	300
Blood test (CBC)	1	250	250
X-ray chest	1	400	400
Total			1,000

Example C: Stock reorder level (dummy figures). If a sample item is used at 20 units per day, supplier lead time is 5 days and safety stock is 30 units, then Reorder Level = (20 x 5) + 30 = 130 units. When the stock register shows 130 units, a new indent is raised.

Example D: Bed Occupancy Rate (dummy figures). For a 50-bed hospital with 1,200 inpatient days in a 30-day month: BOR = 1,200 / (50 x 30) x 100 = 80%.

10. Project: Hospital Workflow & Patient Records Management

Scope. The internship work was organised around a project on hospital workflow and patient records management, covering patient registration, OPD / IPD flow, billing and insurance claim documentation and medical record arrangement, using sample forms and dummy records.

Steps followed. (i) Studied the departments and patient flow; (ii) prepared sample registration and admission forms; (iii) mapped OPD and IPD processes; (iv) prepared sample bills and claim checklists; (v) arranged and indexed dummy medical records; (vi) reviewed the work with the supervisor.

Skills and tools. Patient Registration, OPD / IPD Workflow, Billing & Insurance Claims, MRD, NABH Standards, MS Excel.

Outputs. Process maps, sample forms and checklists, process documentation and this internship report.

11. Case Study Exercise: Long Waiting Time in a Hospital OPD

A case study exercise was completed on a typical administrative problem. The hospital and figures are illustrative and not taken from any real institution.

Part	Details
Background	A 100-bed district hospital has an OPD that serves about 400 patients every morning. Patients complain of waiting 2 to 3 hours for a consultation and again at billing and pharmacy.
Problem observed	Single registration counter; handwritten registration slips; patients arriving without appointments at the same time; billing and pharmacy queues joined at one window; case files searched manually.
Analysis	Mapping the OPD journey showed that registration (about 25 minutes) and billing (about 30 minutes) were the bottlenecks. Most delays came from repeated data entry and missing UHID details, not from the doctors.
Recommendations	(1) Add a second registration counter and a separate counter for follow-up patients. (2) Use a UHID / ABHA-linked registration format to avoid repeat entry. (3) Introduce appointment slots and a token system. (4) Separate billing and pharmacy queues. (5) Display expected waiting time and collect weekly feedback.
Expected result	Shorter waiting time, fewer errors in bills, better patient satisfaction and a more even load on staff.
Lesson learned	Small process changes, supported by accurate records and feedback, can improve patient experience without extra clinical staff.

12. Mock Practical Exercise: Admission to Discharge of an IPD Patient

In the mock practical exercise the student handled the administrative side of one sample (dummy) inpatient from start to finish, using the forms and registers practised earlier.

Step	Task	Document prepared / checked	Department
1	Admission request	Admission slip, UHID check, ID proof and consent form	Registration / IPD
2	Bed allotment	Bed allotment register entry, ward and bed number noted	IPD / Nursing
3	Insurance (if any)	Pre-authorisation request with required documents	Billing / TPA desk
4	Treatment period	Interim bill, medicine indents and issue slips	Billing / Pharmacy
5	Discharge planning	Discharge summary checked for completeness	IPD / MRD
6	Final billing	Final bill, package or itemised, with insurer approval	Billing
7	Discharge and filing	Discharge slip, advice and follow-up date; case file indexed in MRD	IPD / MRD

The exercise showed that every step depends on the one before it: a missing consent form or wrong insurance detail at admission can delay discharge and the final claim.

13. Sample Checklists Prepared

Checklists were prepared as part of the hands-on tasks. They reduce errors and make routine work easier to review.

Admission Checklist	Discharge Checklist
Patient identity and UHID verified	Doctor's discharge order available
Admission form completed and signed	Discharge summary completed and signed
Informed consent for admission obtained	All investigation reports attached
Bed and ward allotted and noted	Medicines and advice explained to patient
Insurance / TPA details recorded	Final bill prepared and approved
Advance deposit receipt issued	Insurance claim documents ready
Allergy and emergency contact noted	Follow-up date and instructions given

Insurance Claim Document Checklist	Patient Safety Checklist
Claim form signed by patient and hospital	Patient wrist-band / identity check done
Copy of insurance card and photo ID	Hand hygiene facilities available
Pre-authorisation approval letter	Medicines labelled and stored correctly
Discharge summary and treatment details	Bio-medical waste segregated in colour-coded bins
Itemised final bill with receipts	Fire exits and equipment clear and working
Investigation and lab reports	Incident reporting form available at the ward

14. Tools and Resources

Learning materials, sample hospital forms and formats, MS Excel and MS Word for registers, bills and documentation, NABH and other reference documents, assignments and mentor feedback were used during the internship.

15. Programme at a Glance

Particulars	Details
Domain	Hospital & Healthcare Administration
Agency	RKNEXORA PRIVATE LIMITED
Minimum hours required	120
Learning days planned	28
Phases	Orientation (Days 1-2); Learning and Practice (Days 3-22); Project and Review (Days 23-28)
Record of attendance	Internship Attendance Log, signed by the supervisor (see Attendance Sheet)

LEARNING OUTCOME

a. Knowledge Acquired

1. Role, structure and functions of a hospital, the types of hospitals and the main groups of hospital departments.
2. The journey of an OPD and an IPD patient and the administrative steps involved, including UHID, admission, transfer and discharge.
3. How patient registration, billing, insurance claims (TPA, cashless, pre-authorisation) and medical records are managed.
4. How hospital inventory and pharmacy are administered, including indent, stock register, expiry checks and reorder level.
5. Basic ideas of quality, patient safety, infection control and the NABH standards.
6. Patient rights, informed consent, confidentiality, key healthcare laws and the importance of ethics and grievance redressal in healthcare.
7. How hospital performance is measured through indicators such as bed occupancy rate and average length of stay.

Glossary of Terms Learned

Term	Meaning
OPD	Out-Patient Department - care of patients who are not admitted
IPD	In-Patient Department - care of admitted patients
MRD	Medical Records Department - stores and protects patient records
UHID	Unique Hospital ID - one identity number linking all visits of a patient
ABHA	Ayushman Bharat Health Account - a digital health ID
TPA	Third Party Administrator - handles insurance claims between hospital and insurer
Pre-authorisation	Insurer's approval taken before planned treatment
Cashless	Treatment where the insurer pays the hospital directly
Discharge Summary	Summary of a patient's treatment given at discharge
Informed Consent	Patient's permission after being told about a procedure and its risks
NABH	National Accreditation Board for Hospitals & Healthcare Providers
Patient Safety	Prevention of harm to patients during care
Indent	A request for medicines or materials from the stores
FEFO	First Expiry, First Out - issue items that expire first
Grievance Redressal	Process of receiving and resolving patient complaints
Triage	Sorting patients by urgency of need
BOR / ALOS	Bed Occupancy Rate / Average Length of Stay

b. Skills Learned

Technical Skills	Professional Skills
Patient registration and admission forms	Communication and courtesy
Billing and insurance claim documentation	Empathy and patient handling
Medical record filing and retrieval basics	Confidentiality and ethical conduct
Inventory and pharmacy stock registers	Teamwork and coordination
Quality and patient-safety checklists	Time management
MS Excel and MS Word for documentation	Documentation and report writing

These skills were developed through repeated practice and daily documentation. Careful form-filling improved accuracy, process mapping improved analytical thinking, and writing remarks and reports improved communication.

c. Observed Attitude and Gained Values

1. Confidentiality and respect: patient information and dignity must always be protected.
2. Empathy and service: administrative work exists to support patient care.
3. Accuracy and responsibility: a small error in a record or bill can affect a patient.
4. Discipline and punctuality: regular reporting at fixed timings and daily attendance records.
5. Teamwork and integrity: cooperating with colleagues and keeping honest records.

d. Challenges Faced

Challenges Faced	How They Were Handled
Understanding unfamiliar medical and administrative terminology.	Used a glossary of terms, reference material and mentor guidance.
Handling the sensitivity of patient information.	Used only sample and dummy records and followed confidentiality rules.
Understanding the billing and insurance claim process.	Studied sample bills and claim checklists and reviewed them with the supervisor.
Linking quality standards such as NABH to daily routines.	Used checklists and worked examples to connect each standard to a routine task.
Maintaining consistent documentation.	Used daily log entries and reviewed feedback to improve documentation progressively.
Managing time between practice, documentation and report writing.	Divided work week by week and used review days for corrections.

e. Career Pathways

The knowledge gained supports further study and careers such as Hospital Administrator, Patient Relations Executive, Medical Records Officer, Billing and Insurance (TPA) Executive, Pharmacy / Stores Executive, Quality Coordinator (NABH) and Front Office Executive. Related higher-study options include MBA / PGDM in Hospital Administration or Healthcare Management, and Master's programmes in Health Administration or Public Health.

CONCLUSION

The internship in Hospital & Healthcare Administration was a valuable learning experience that integrated academic knowledge with practical activity. The programme achieved its objectives through structured learning, guided practice, assignments, supervisor review and documentation.

The student gained a clear understanding of how hospital administration supports patient care - through registration, OPD / IPD management, billing and insurance, medical records, inventory and pharmacy, and quality, law and ethics - and acquired practical familiarity with forms, registers, checklists and process documentation. The experience has strengthened the student's employability and provides a foundation for further study and work in hospital administration, health services management and related fields.

Reflection

Looking back, the most useful lesson of the internship was that good patient care depends on careful administration behind the scenes. Moving from concept to practice to documentation made abstract ideas concrete, and supervisor feedback showed where understanding was weak. The habit of recording each day's work also made it easier to see progress over the weeks.

Objectives and Achievement

Objective	How it was achieved
Understand hospital administration	Concept sessions, study of hospital structure and glossary of terms
Study departments and patient flow	Organisation chart study and OPD / IPD process mapping
Learn patient-care administration	Sample registration forms, bills, claim checklists and record filing
Understand support services	Sample stock registers and expiry / reorder checks
Learn quality, law and ethics	Safety checklists, NABH study, patient-rights and grievance exercises
Develop professional discipline	Daily attendance, supervisor review and report writing

Future Scope

The knowledge gained can support further study of hospital management and careers in hospital administration, billing, medical records and patient relations. With India's growing healthcare sector, digital health records and insurance coverage, trained administrative professionals are increasingly needed.

Suggestions for Improvement of the Programme

1. Include more visits and live demonstrations in hospital settings, with appropriate permissions.
2. Continue regular mentor feedback and progress reviews.
3. Provide more project-based and collaborative activities.
4. Encourage students to maintain detailed daily documentation.

References

1. Kolb, D. A. (1984). Experiential Learning: Experience as the Source of Learning and Development. Prentice Hall.
2. University Grants Commission (UGC). Internship guidelines for higher education institutions.
3. Ministry of Education, Government of India. National Education Policy 2020.
4. National Accreditation Board for Hospitals & Healthcare Providers (NABH). Accreditation standards for hospitals.
5. Government of India. The Clinical Establishments (Registration and Regulation) Act, 2010.

6. Government of India. The Consumer Protection Act, 2019, and the Digital Personal Data Protection Act, 2023.
7. Ministry of Environment, Forest and Climate Change. Bio-Medical Waste Management Rules, 2016.
8. National Health Authority. Ayushman Bharat PM-JAY and Ayushman Bharat Digital Mission (ABHA) guidelines.
9. RK NEXORA internship programme handbook.

ATTENDANCE SHEET

The Internship Attendance Log issued by the IPO, with the supervisor's signature for each day, is to be attached on this page.

[Attach the signed Internship Attendance Log here]